Preventive Care in the Context of Men's Health

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Abstract

This study assessed the effectiveness of a health intervention, in diabetes education and risk assessment, to motivate men to take positive action, within the framework of the Health Belief Model. The worksite-based intervention targeted 525 men aged between 40 and 65 years from 27 organizations in the south-west of Western Australia in 1999/2000. Men identified as high risk were referred to their general practitioner. The adopted strategy had a number of strengths, namely that the health risk assessment was topic-specific, personalized, conducted on the worksite and combined with an educational component and a medical follow-up. The focus groups component of the study gave further understanding as to why the adopted strategy had been appropriate for men's needs.

Keywords

diabetes prevention, health assessment, Health Belief Model, worksite intervention

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Background

THE CONCEPT OF 'Men's Health' emerged as a distinct social and health issue in Australia and elsewhere during the 1990s, and took its place on the national health agenda as an area requiring special consideration, particularly for men in rural and regional Australia (House of Representatives, 1997; Lawrence, 1995). To improve the health of rural men, several elements can be assumed to be intertwined: the reluctance of men to confront health issues proactively; their ambivalence about consulting general practitioners; the appropriateness of the worksite as a venue to facilitate men's access to advice on health matters; and the application of a health promotion model likely to encourage positive action towards improved health.

There is a small but growing literature on men's health attitudes and behaviours and how these differ from women's. In a study of attitudes to coronary heart disease among employed people in Ireland, men were generally less motivated to change health behaviours than women, older men thinking it was too late and younger men thinking it was too soon. Barriers to change identified included the difficulty of changing routines and lack of personal discipline. Motivation to change was principally undermined by scepticism about apparently contradictory medical advice. Men expressed a need for specific education and clear advice from doctors: 'Until you go to a doctor and you're told that you have high cholesterol or whatever, it's only then you'll have to get down to doing something about it' (Gabhainn, Kelleher, Naughton, Carter, Flanagan, & McGrath, 1999, p. 624).

Most reasons for reluctance to visit a GP revolve around the 'macho' social conditioning of males (Huggins, Somerford, & Rouse, 1996). It is argued that men view seeking help as a sign of weakness, and feel it is inappropriate to expose vulnerabilities or to disclose intimacies to other men, and sometimes even to their doctor. When symptoms are manageable, it is preferable to 'soldier on in silence'. When they become severe, it is acceptable to seek help, especially when 'pushed' by one's partner, but the delay means that males tend to visit the GP for life-threatening conditions for which they are hospitalized (Huggins et al., 1996). Debate about the claim that men delay seeking help for medical conditions (Fletcher, 1995) reflects the fact that little is empirically known about men's views about their health concerns or how they would improve existing health services, with rural men in particular being overlooked (Fletcher, 1995).

The perception that general practitioners are the most reliable, credible source of health information is commonly held (Oldenburg & Owen, 1995) and has been empirically demonstrated in rural Australia (Humphreys, Rolley, & Weinand, 1993). Although men see general practitioners as their principal health advisers, they visit less frequently than women and express reservations about the lifestyle advice they are given (Ashenden, Silagy, & Weller, 1997). Preventive care is argued to be underutilized by general practitioners in the primary care setting (Ashenden et al., 1997; Oldenburg & Owen, 1995). Limited time in consultations coupled with remuneration problems are further compounded by the lack of training provided to medical practitioners in preventive care, such as counselling and education skills needed to encourage lifestyle modification by patients, and the competencies needed in moving from a paternalistic model of determining treatment options to one of shared decision making between GP and patient (Charles, Whelan, & Gafni, 1999; Oldenburg & Owen, 1995; Towle & Godolphin, 1999).

If the opportunity to undertake health prevention is constrained in the GP setting, particularly for men who visit less frequently, how can at-risk males be accessed with a view to addressing modifiable behaviours that contribute to disease? Gabhainn et al. (1999) advocated focused workplace health interventions to meet this challenge. Such a strategy was seen as appropriate for older men (>45 years), who were generally well informed but unlikely to perceive themselves as willing or able to take action.

Models of change in health behaviour offer some insight into designing and evaluating worksite health promotion interventions. One such model, which can accommodate multiple intervention components such as general education and personalized assessment, is the Health Belief Model (HBM) (Nutbeam & Harris, 1998).

The HBM is generally acknowledged as the

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genesis of a systematic, theory-based approach to research in health behaviour and is the most frequently cited and researched model (Harrison, Mullen, & Green, 1992). It comprises five main components which can be practically applied in a health promotion intervention: *perceived susceptibility*, assisting an individual to recognize their susceptibility to a disease, that is an appreciation of risk factors and a self-assessment on these factors; *perceived severity*, assisting an individual to understand the seriousness of the consequences of a particular disease; *perceived benefits of taking action*, providing a forum in which an individual can assess the benefits which should flow from taking preventive or early intervention action; *perceived barriers to taking action*, providing a forum in which an individual can identify and analyse the impediments to taking action and how these might be overcome; and c*ues to action*, providing a discrete, concrete, low-risk first step to initiate action, capitalizing on the momentum generated by the awareness-raising in the earlier components.

A workplace health intervention focused on diabetes seems appropriate for many reasons: diabetes is now the fifth health priority in Australia and the prevalence of undiagnosed diabetes is estimated to represent half the total prevalence of diabetes (Colagiuri, Coligiuri, & Ward, 1998; Commonwealth Department of Health and Aged Care, 1999). Early identification of risk factors provides opportunities for prevention or delaying the development of Type 2 diabetes. Both national and state governments acknowledge the disadvantage faced by rural people in availing themselves of all aspects of diabetes management, from screening to regular assessment and education, yet the need for such access is apparent (Commonwealth State Diabetes Forum, 1999; Western Australian Diabetes Services Taskforce, 1999). In the rural south-west of Western Australia, the local reported prevalence rate of diabetes (5.6 percent) was nearly double that reported in the state (2.6 percent) and the prevalence of diagnosed diabetes in men compared to women was nearly three times higher (Aoun & Stone, 1997).

It is in this context that a brief health intervention in diabetes education and screening, for men aged 40–65 years, was delivered to organizations in the rural south-west of Western Australia in 1999–2000.

Study objectives

The overall aim of the study was to assess the effectiveness of a disease-specific intervention on motivating men to visit a GP or take other health-enhancing steps when provided with personalized information about their health status. The study had also been designed to assess the utility of the Health Belief Model in developing the health intervention.

The qualitative research objectives included gaining a general understanding of blue-collar men's views of health, their understanding of the relationship between various risk factors and heart disease, cancer and diabetes, their major health concerns and their actual and preferred sources of information and influence with respect to health. These results were useful in informing the development of the health intervention.

Method

There were two components to the study. The first component consisted of qualitative research conducting focus groups with men, aged 40–60 years, working in the south-west of Western Australia. The second component was the delivery of a health intervention in diabetes to men aged 40–65 years working in organizations in the same geographical location.

Focus groups

Four focus group discussions were held with blue-collar men in two age groups: 40–49 years and 50–60 years. This number of groups was sufficient to allow all relevant issues to emerge. In fact, virtually all major themes emerged after two groups. Two of the groups were with metropolitan men, and two with men in the rural south-west of Western Australia. This allowed the identification of any issues that may have been different for rural and metropolitan blue-collar men. The groups in the south-west were recruited by a professional market research recruitment agency via random selection from the local telephone directory, and in the metropolitan area from the agency's database. A brief questionnaire was used to select qualified respondents. Men with a chronic illness were excluded from the groups. None of the men was involved in the health intervention described below.

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The intervention

Liaison with industries A list of relevant industrial organizations (N = 41 of various sizes) was obtained from the South West Development Commission and a media release was issued describing the project and inviting organizations to participate. A phone call to introduce the project to the manager of the organization was followed by a detailed letter to the Occupational Health and Safety Officer, Health Nurse or another identified contact suggested by the manager. The letter explained the phases of the project and enlisted the organization's participation and support in contacting the men and organizing the date and venue for education and screening. To help with this, a flyer was designed to be attached to the workers' pay slips or pinned on notice boards, inviting them to attend and to register their response through the tear-off reply section of the flyer. Posters on the Diffuse Diabetes Campaign (coinciding with the project) were supplied by Diabetes Australia and were sent to advertise the project in the worksites well beforehand.

Education and screening programme The project was undertaken by staff from the Western Australian Centre for Rural Health and Community Development in collaboration with the Diabetes Educator from the Bunbury Health Service. The programme of the educational sessions was tailored to the needs of men in the target group 40–65 years following the guidelines of Diabetes Australia. The presentations included information on diabetes, who is at risk, what are the symptoms, what are the long-term problems from diabetes, ways to reduce the risk factors and healthy lifestyle recommendations.

Three sets of data were obtained from participants after they had attended the educational session:

1. The risk assessment test—which included a list of symptoms of diabetes, family history, ethnic background, history of heart disease or a previous borderline high blood sugar test, and involved measurement of blood pressure, waist circumference and height and weight by screening staff.
2. Questionnaire on the extent and nature of GP service utilization.
3. Evaluation questionnaire—which obtained participants' feedback on the educational session, including whether it was interesting, useful, easy to follow, the least and the most useful components, whether it motivated them to make any changes to their lifestyles and the extent to which it affected their awareness of the consequences of diabetes, of whether they are at risk of developing diabetes and what they can do to reduce their risk of getting diabetes.

The screening team consisted of a diabetes nurse educator, a community nurse, a medical student and the project researcher. Men were identified to be at risk if they were 40–49 years and scored two or more risk factors, or they were 50 years and over with one or more risk factor. Blood pressure was a risk factor if it was greater than 140/90 mmHg or was being treated with medication. The screening team took weight and height measurements and men checked them against a weight-for-height chart. They could be in one of four zones: obese, overweight, healthy or underweight. The chart indications were checked against and coincided with the Body Mass Index (overweight having a BMI>25 and obese a BMI>30). The waist circumference measurement was another indication of being overweight (100 cm or over).

Follow-up At the end of the screening session, men who were identified to be at risk were given a letter of referral to their GP signed by the diabetes educator. The letter listed the risk factors found and recommended that the GP do a medical review and a venous plasma glucose test to screen for Type 2 diabetes. All GPs in the geographic areas of the project were informed about the project. To take part in the follow-up, men identified as at risk were invited to sign a consent form for the project team to contact them regarding the outcome of their visit to the GP. The participants were followed up one month after the screening session, either by sending them the questionnaire with a reply-paid envelope or by a telephone call. In larger organizations, Occupational Health and Safety Officers assisted in distributing and collecting the follow-up forms. For some respondents, up to three contact attempts were needed to complete the follow-up questionnaire on the outcome of their visit to the GP.

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The follow-up questionnaire collected information on whether the respondent had visited a GP as a result of the screening session, whether a blood glucose test was done, whether the result of the test showed if diabetes was likely, unlikely or unclear and requiring further investigation, what other tests the GP arranged, whether the GP had provided any healthy lifestyle advice and whether the respondent was asked to attend another appointment and how soon. Those who did not visit a GP were asked if they still intended to and if not, to specify a reason.

Results: focus groups

The tapes were transcribed and content analysed for major themes. The two moderators (RD and GE) independently developed themes and cross-matched them for consistency. The major themes were mapped against the Health Belief Model in terms of perceived susceptibility, cues and barriers to action. There were no major differences between rural and metropolitan men with respect to the identified themes. Thirty men participated in the four focus groups.

Perceived susceptibility

Awareness of differences in health statistics for men and women Men were generally aware that women live longer. When asked why, they generally agreed that they just do not take the same interest in their health as women do, that men indulge in more unhealthy behaviours and they defer seeking action that could have prevented premature death: 'we self destruct' . . . 'If he'd gone to the doctor, he'd probably still be alive' (comment on a workmate's sudden death).

Understanding of health and risk factors Most had a general awareness of the main causes of death in Australia (cardiovascular diseases and cancers), and hence these constituted their main health concerns. They also appeared aware of many of the main risk factors associated with these: heart disease with high cholesterol, high fat diets, physical inactivity, being overweight and high blood pressure; bowel cancer with a low fibre diet; lung cancer with smoking; high blood pressure with stroke; and skin cancers with exposure to sunlight. However, there was little knowledge of risk factors associated with what they called '*sugar* diabetes', and there was low appreciation of the general benefits of physical activity across all three areas of heart disease, cancer and diabetes.

Although aware of the links between lifestyle factors and diseases, men's understanding of the links appeared somewhat superficial and not always sufficiently understood to take action. Knowledge of diabetes was low. Very few knew of an association with body weight and the association of Type 2 diabetes with heart disease.

Cues to action

Reaching a decade acts as a cue to assessing one's life in many areas, including health status: at 40 or 50, 'you get a wake-up call . . . you realize you're not bullet-proof'. Men in the 40–49 years group stated that 'turning 40' and men in the 50–60 years group stated that 'turning 50' was a major factor in reassessing their health and fitness. The difference seemed to be primarily one of facing a *cosmetic* decline at 40, especially in fitness and appearance (i.e. putting on weight), versus facing one's mortality in their 50s.

An increased awareness of their mortality seemed due largely to an awareness of friends, acquaintances and workmates dying of heart attacks or cancer in their late 40s and early 50s. Discussion at the workplace or elsewhere then often focuses on 'going for a check' and comparing outcomes. The men stated that test results can have a motivating effect, as they provide concrete evidence—often in the absence of symptoms—that a change in behaviour is required. Hence an appropriate strategy would seem to be to engage men in a screening programme and then use the results to motivate action.

Factors influencing action For most men, the most influencing factor (their 'wake-up call') was a close friend, partner or fellow employee succumbing to some disease. The closer the relationship and the more sudden or quick the death or disablement, the more impact: 'one of our crew had a heart attack and a triple bypass . . . this convinced all the crew to have a cholesterol test and a treadmill test'. One respondent stated that he came to the research group

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because he had recently 'lost a lot of mates' and was now interested in men's health.

Barriers to action

In keeping with research in Australia (Egger, 2000; Pinnock, O'Brien, & Marshall, 1998), and recent US research (Sandman, Simantov, & An, 2000), 'men don't find it easy to talk about health or go to the doctor'. They 'tend to soldier on and be the hero', rather than take early action, with the result that 'damage is done before they seek help'. Many men feel embarrassed about going to a doctor for minor ailments, seeing this as contrary to their perceived cultural expectations of men putting up with and not complaining about pain (i.e. not being seen as 'a wuss'). They are—or feel—further restricted by time pressures (i.e. having to take time off work), and the knowledge that the total time (i.e. actual consultation plus waiting time and travel time) would be quite considerable.

Some men expressed disappointment or frustration that their GP, on a previous visit, did not provide more specific directions for lifestyle changes: 'he just said take it easy and come back and see me in six months'.

Nevertheless, GPs were frequently mentioned in discussion as a potential and desirable point of contact. It appears that a GP is still a respected source of information from whom a man can seek information without showing a form of 'weakness' to other men.

However, men who came to the focus groups express great satisfaction with the opportunity to talk about health issues with other men (Egger & Mowbray, 1993). Worksite health checks may provide an opportunity for these needs to be met in an acceptable way.

Attitudes to health checks Given that men are reluctant to go to the doctor unless there's 'something really wrong', it is not surprising that men are reluctant to go to a GP for a health check when they are symptom-free. However, these men said that they would undergo a health check if offered at the worksite and if there is a supportive work culture. Worksite screenings therefore may attract an otherwise disinterested audience, or an audience that seeks 'permission' to engage. It appears that tests for specific health topics (like diabetes) might be more likely to attract compliance than general health checks.

Conclusions from focus groups

The focus groups suggest that worksite health intervention would provide an appropriate opportunity to reach and impact men in the 40+ years of age, especially if it was based on a specific health issue. If men are to be referred to their GP for follow-up, it is essential that GPs be aware that men would appreciate concrete lifestyle behaviour advice, rather than simply being told to lose weight or eat more healthily.

Health intervention

Education and screening

Twenty-seven organizations participated in the health intervention (66 percent response rate). Organizations had different periods of the year where a training programme could not be accommodated depending on the nature of their work (production, processing, farming, agriculture, timber, mining and manufacturing). Hence the screening programme spanned a period of nine months.

The number of men voluntarily participating in the education and screening was 525, with 446 in the target group of 40 to 65 years. Men were very appreciative of the education provided in terms of presentation, content and relevance: 94–100 percent found the education session interesting, useful and easy to follow; 28 percent found that most of the information was new to them and 67 percent found some of it new.

The screening component revealed that the susceptibility of this group to developing diabetes was significant. In the group 40–49 years, 48 percent were at risk (having two or more risk factors) and in the group 50–65 years, 85 percent were at risk (with one or more risk factors), with 64 percent at risk overall (see Table 1). Of the modifiable risk factors, 62 percent of men were overweight and 33 percent had high blood pressure. Of the non-modifiable risk factors, 26 percent of men had a family history of diabetes and 8 percent were from a predisposing ethnic background (see Figure 1). In addition, 4 percent had a previous borderline high blood sugar test and 6 percent had a history of heart disease.

GP service utilization

The extent to which GP services had been utilized by this group of men in the past year

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Table 1. Risk status of men by age

Age group	*Low risk*		*High risk*		*Total*	
	N	*%*	*N*	*%*	*N*	*%*
40–49 years	128	52	118	48	246	55
50–65 years	31	15	169	85	200	45
Total	159	36	287	64	446	100

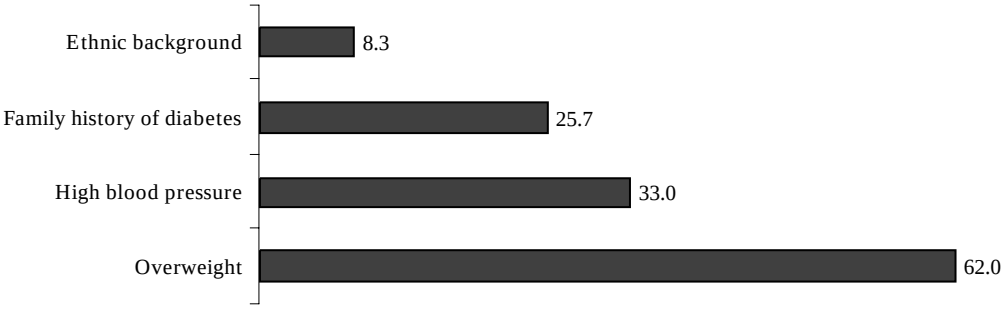


Figure 1. Percentage of participants with risk factors

(prior to the intervention) was similar to that of the general population of men in the area (76 percent). These visits were mainly due to routine checks initiated by the employer (40 percent) with the remainder due to symptoms, prescription renewal, test results or needing medical certificates. Barriers to accessing GPs have not been a major issue for 68 percent of these men, with 'not considering that their problem was serious enough' being the most frequently stated reason for not visiting their doctor (18 percent) (see Table 2).

Follow-up

While all men at high risk were advised to see their GP and given a letter of referral, 91 percent consented to be followed up and 60 percent responded to the follow-up survey. Of these respondents, 76 percent visited their GP, and in 87 percent of cases had blood glucose tests as advised in the referral letter. The feedback of men on the results of these tests in the follow-up survey revealed that 2 percent had undiagnosed diabetes and that 6 percent had impaired glucose tolerance. Other health problems

Table 2. Factors inhibiting attendance at GPs

Factors inhibiting attendance	*Percentage*
Nothing prevents from visiting GP	68
Problem not serious enough	18
A lot of time waiting	7
Not enough time to attend	6
Surgery times inconvenient	5
Can't be bothered	3
Cost a problem	2
Other (e.g. difficulty in securing timely appointments)	5

Total does not add up to 100% due to multiple responses

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emerged from these tests: 32 percent of men had raised blood pressure and raised cholesterol at a level requiring medication, and 40 percent of those who visited their GP were asked to return within two weeks to two months so that their risk factors could be monitored. When it came to getting advice from their GP, 49 percent of men visiting their GP had received lifestyle advice, mostly with respect to losing weight and exercising more.

Mapping the health intervention against the Health Belief Model

The diabetes education and health risk assessment addressed the five components of the Health Belief Model as illustrated in Table 3. The focal outcomes were explicitly short term with an emphasis on increased awareness of risks, a commitment to change and the intermediate behaviour of visiting a GP, rather than the long-term reduction of risk factors. A number of the men's comments, made during the health intervention and recorded in the evaluation questionnaire, are included in Table 3 in italics.

Discussion and conclusion

This health risk assessment has revealed that the extent of susceptibility of this group of men to developing diabetes was significant (64 percent). The intervention was also successful in engaging men in the preventive concept of seeking care and getting them to attend their GP to check the extent of their risk. The strategy adopted for this project had a number of strengths, namely that the health risk assessment was conducted in the worksite, was topic-specific and personalized and was combined with an educational component and a medical follow-up. Early detection was the main focus of the screening, thus giving men the opportunity to prevent the disease if they modified their lifestyle behaviour. The HBM was a useful conceptual framework within which to design and evaluate the intervention.

Overall, the quantitative research results reinforced the qualitative results. The group discussions indicated that an intervention at the worksite was appropriate for blue-collar men, that a specific health issue was more likely to be successful than a general health appeal and that test results would be likely to motivate further action, including a visit to their GP on the basis of an adverse test result. Furthermore, the focus groups confirmed that diabetes education was urgently required among this age group of men, as men have very little knowledge about risk factors for diabetes and are unaware of the growing prevalence of this disease and the debilitating effects. This is further emphasized, in the quantitative study, by the most frequently reported reason for not visiting a GP as being that men 'did not consider their problem was serious enough'. Pinnock et al. (1998) support this finding by stating that a major barrier to health action by men is that they view potentially serious symptoms as simply signs of growing old, reflecting their lack of knowledge about health matters in general.

Men need concrete advice on how to translate knowledge into action in their daily lives. From the focus groups, men's attitudes as a result of previous encounters with GPs confirmed the need to involve GPs in the intervention so that they were prepared to offer men concrete advice and specific, attainable behavioural recommendations. However, about half of the men visiting their GPs reported getting some lifestyle advice, but not enough to act upon. It is worth noting that findings from a workshop, with health professionals including GPs, that followed the intervention suggested that advice, which would induce a behavioural change, is extremely difficult considering the time constraints in rural practice compounded by a lack of GP training in lifestyle education and counselling. Thus this topic would be more appropriate for other health professionals to undertake in-depth, raising issues of support strategies within health services for men at risk of developing diabetes.

Changing times: The climate is right for preventive action

The current research suggests that many Australian blue-collar men still think that men have to be 'heroes' and that it is still a bit 'sissy' to show any worry about your health, but the mood appears to be changing. They readily acknowledge they have a problem in not being able to 'open up' to other men, and are grateful for the opportunity to be able to talk about it in the research groups and participate in the educational and screening programme:

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Table 3. Intervention mapped against HBM

Component	*Application in intervention*
Susceptibility/awareness of risk increased greatly for 64 percent of men *'Finding out I was at risk—some of the factors I wasn't aware of'* *'how susceptible I am with my lifestyle'*	• Education: in a group session, informing participants about risk-factors • Screening: – Self-assessment of risk factors such as family history using a questionnaire – Objective individual assessment of physiological measures of risk such as blood pressure and weight
Severity/awareness of consequences increased greatly for 75 percent of men *'Now I know two things that make you go blind!'* *'the consequences of doing nothing about it'*	Education: in a group session, informing participants about potential consequences of the disease
Benefits/awareness of reducing risk increased greatly for 71 percent of men *'diet and exercise—getting information to reinforce action'*	Education: • In a group session, informing participants about options to reduce risk via lifestyle changes such as in diet and exercise • Having available print material such as brochures on healthy eating
Barriers/impeding change ***Regarding lifestyle*** *'I love beer and meat'* *'I am too busy'* ***Regarding GP access*** *'delays in securing appointments'*	Discussion: • In group education session, providing opportunity for discussion about impediments to change • For individuals identified as high risk, interacting informally on one-to-one basis with Diabetes Educator during discussion of results of risk-factor questionnaire
Cues to action • **31 percent of all men were highly motivated to change lifestyle and 63 percent were partly motivated** • **76 percent of respondents to follow-up survey, visited their doctor** *'the session has confirmed it's time to act and not wait for the day',* *'timely reminder of what we know we should be doing'*	• Acknowledgement of awareness and commitment: asking participants to indicate to what extent they had learned something and were motivated to make lifestyle changes • Referral: giving individuals at high risk of developing the disease a letter of referral to GP requesting further investigation • Anticipation of follow-up: informing participants given a letter of referral that, with their consent, they would be contacted about the outcome of their GP visit

A very good idea to have several men gathering to hear the same message and possibly helping each other through any difficulties if and when it occurs.

Sharing the learning experience with workmates makes it more comfortable.

Bloody good idea to participate in such a men's group.

It appears that greater acceptance of occupational health initiatives has resulted in greater acceptance of men taking an interest in health in general.

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